

OUTLINE OF EVIDENCE – MR CORY LEA SHEPHERD (APPELLANT)

Served pursuant to direction 2 of the Further Directions Order (3) dated 19 August 2026. Not filed in the Industrial Registry.

I will give evidence of the following. Paragraph references are to the notice to admit facts served on the Respondent on 28 August 2026 (the Notice), whose Annexure A holds the documents referred to.

- 1. Employment and the role.** I commenced at Logan Hospital Switchboard on 25 March 2019 as a casual, became permanent from 3 March 2021, and worked full-time hours from 16 October 2023 on my application of 31 August 2023, as a continuous shift worker. The duties: emergency response notifications distributed to the response groups under the code procedures, "strictly adhering to protocols and timeframes"; the pager and on-call records on which the hospital's emergency paging depends; the call queues; and some 200 to 300 calls a shift, including urgent clinical handover, distressed patients and families, and complaints. [Notice ¶¶ 1–16, 26–38]
- 2. Sleep and fatigue.** My sleep on rotating shifts across 2023 and 2024 and what I did about it, including seeing my general practitioner on 16 November 2023 about poor sleep with shift work, when melatonin and temazepam were prescribed. The shifts of 17 and 18 March 2024: where I lived, my travel time each way, finishing at 23:00 and starting at 06:00, the rest and sleep actually available, the emergency codes I handled on 18 March, and the sick leave I took on 19 March. The fatigue concerns I raised and when; sending the fatigue toolkit to Ms Taylor and receiving no response; my request of 8 April 2024 and the refusal of 1 May 2024; and that before 30 June 2024 I was given no fatigue training, no fatigue or psychosocial risk assessment of my rostering, and no fatigue management process. [¶¶ 17–25, 211–256, 263–268]
- 3. Rostering: lates then earlies.** The pattern on my roster line of late shifts followed by early shifts, and of nights followed by days off and then nights again, which I raised on 16 April 2024; that rostering errors on my line recurred across successive roster periods; when I identified each and to whom; and Ms Reese's acknowledgement of 10 May 2024 of "a few rostering errors made by Chloe with regards to Cory's line in past rosters". [¶¶ 211–223]
- 4. Pay and entitlements.** The public holidays I was not rostered to work between February and April 2024 and the difference that made to my pay; that Payroll identified the errors on 3 May 2024 and directed my manager to submit an AVAC for each fortnight; that I could not submit one myself; what I did and was told between 3 and 28 May 2024; and the validation of claims older than three months I was asked to sign on 28 May 2024. The special pandemic leave application of February 2024: what I attached, the two declines, and the approval on the same material. [¶¶ 114–142, 182–210]
- 5. Management and the rules.** That from 18 July 2023 operators could no longer correct database entries and the Contact & Number Changes book was removed, so corrections waited on Ms Stibbard or Ms Taylor, including overnight, at weekends and on public holidays; the directives of 15 April, 19 April and 9 May 2024, and that I was not consulted before any of them; what I saw at the console between 2 and 8 May 2024 when calls reached the wrong teams; that I did not know Ms Taylor's hours, my request of 15 May 2024 that she state them, and the direction to retract that email; the communication book entry of 6 June 2023 and its removal; the concerns I raised on 7 August 2023 and the responses; my complaint of 13 May 2024, later determined to be a public interest disclosure (content not set out); and my interest in becoming the Switchboard union delegate and what followed. [¶¶ 39–113, 143–181, 283–299]
- 6. My health before June 2024.** That before 18 June 2024 I had never been diagnosed with or treated for depression. My general practitioner's record of 16 November 2023 notes poor sleep with shift work, that I could not do shifts without a good sleep, no psychological illness such as depression or psychosis, and mood good. The ADHD and anxiety noted in my history in October 2022 and the specialist referral renewed on 16 May 2024, and what each concerned. [General-practice records disclosed by the Respondent]
- 7. Onset, presentation and diagnosis.** The symptoms I experienced from about 18 June 2024 and their effect on my sleep, concentration, self-care and relationships. Attending Dr Slawinski on 28 June 2024, whose note records "stress at work" and "upset by people not following rules", and Dr Hawes on 1 July 2024, whose note records "work stress", that "they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies", and "they don't listen to his complaints". Lodging my application for compensation on 1 July 2024. Dr Hawes's work capacity certificates of 1 July and 8 September 2024 and his referral to a psychiatrist. My first consultation with Dr Krishnaiah on 24 October 2024, on that referral, and my treatment since. The matters at paragraphs 2 to 5 above are the matters I reported to each doctor, as their records show. I offer no clinical or diagnostic opinion.
- 8. After 18 June 2024.** Meetings scheduled while I was on certified leave; that my employment was ended under abandonment provisions in October 2024 while I held current medical certificates, and that I was subsequently reinstated; my return to work on reduced hours and the leave I have taken since; and that my Queensland Health email account was restricted, so that I cannot now produce from my own records the emails I sent and received at work.
- 9. Documents.** The rosters and payslips for the relevant periods; my emails and messages with my line manager and the Director; the leave and payroll records produced by the employer; the role description; the general-practice records and certificates referred to above; and the letter of Metro South Hospital and Health Service of 5 June 2026 (ref K-LM26/729).

This outline states the topics on which I will give oral evidence. It is not a statement of evidence and is not verified.